

An Evidence-Based Intervention to Address Traumatic Events Among Hospice Nurses

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Background

- Nurses working with high acuity patients frequently experience work-related traumatic events leading to burnout (Cross, 2019; Keidel, 2002).
- Approximately 60% of hospice and palliative nurses, while 20% of these nurses actively seek less stressful nursing jobs (Parker, 2019; Vossell, 2021).
- Burnout is defined as feeling of exhaustion and disengagement, which negatively impact nurses, patients, and healthcare organizations (Al-Majid, 2018; Curry, 2021).

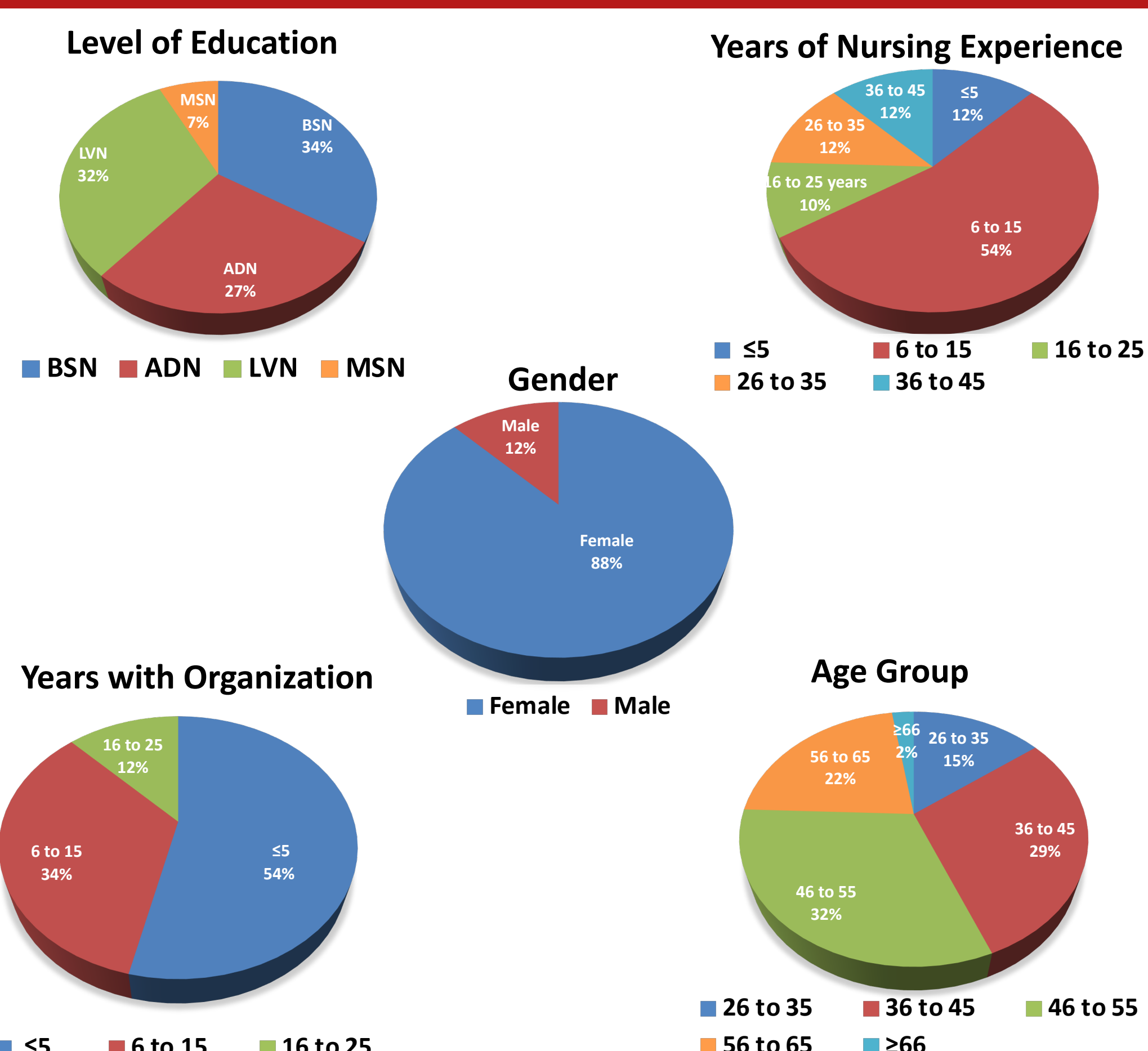
Problem Statement

- Fifty seven percent of nurses in the project's Hospice Agency experience burnout and only 86% intend to stay in palliative care for the next 12 months (Willis Towers Watson Survey, 2022).
- Implementing strategies to attenuate burnout is imperative to support these nurses and decrease nurse turnover.

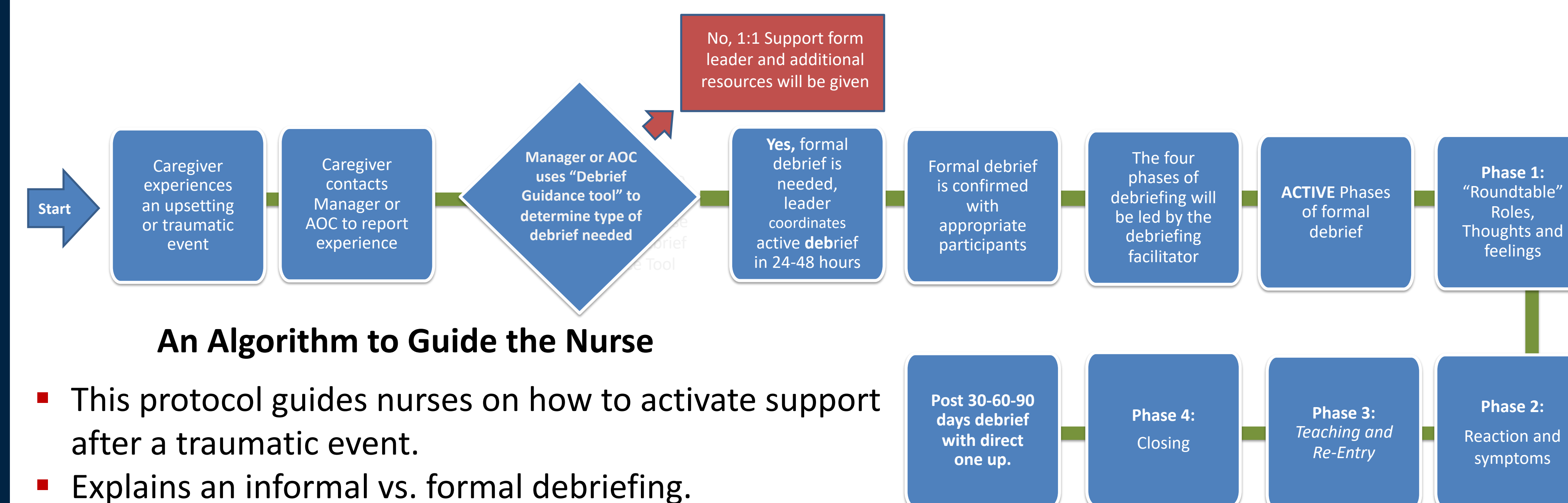
Purpose Statement

The purpose of this Doctor of Nursing Practice project was to examine if a debriefing protocol activated following a job-related traumatic event would decrease burnout, exhaustion, and disengagement among nurses working in a Hospice Agency in Southern California.

Demographics

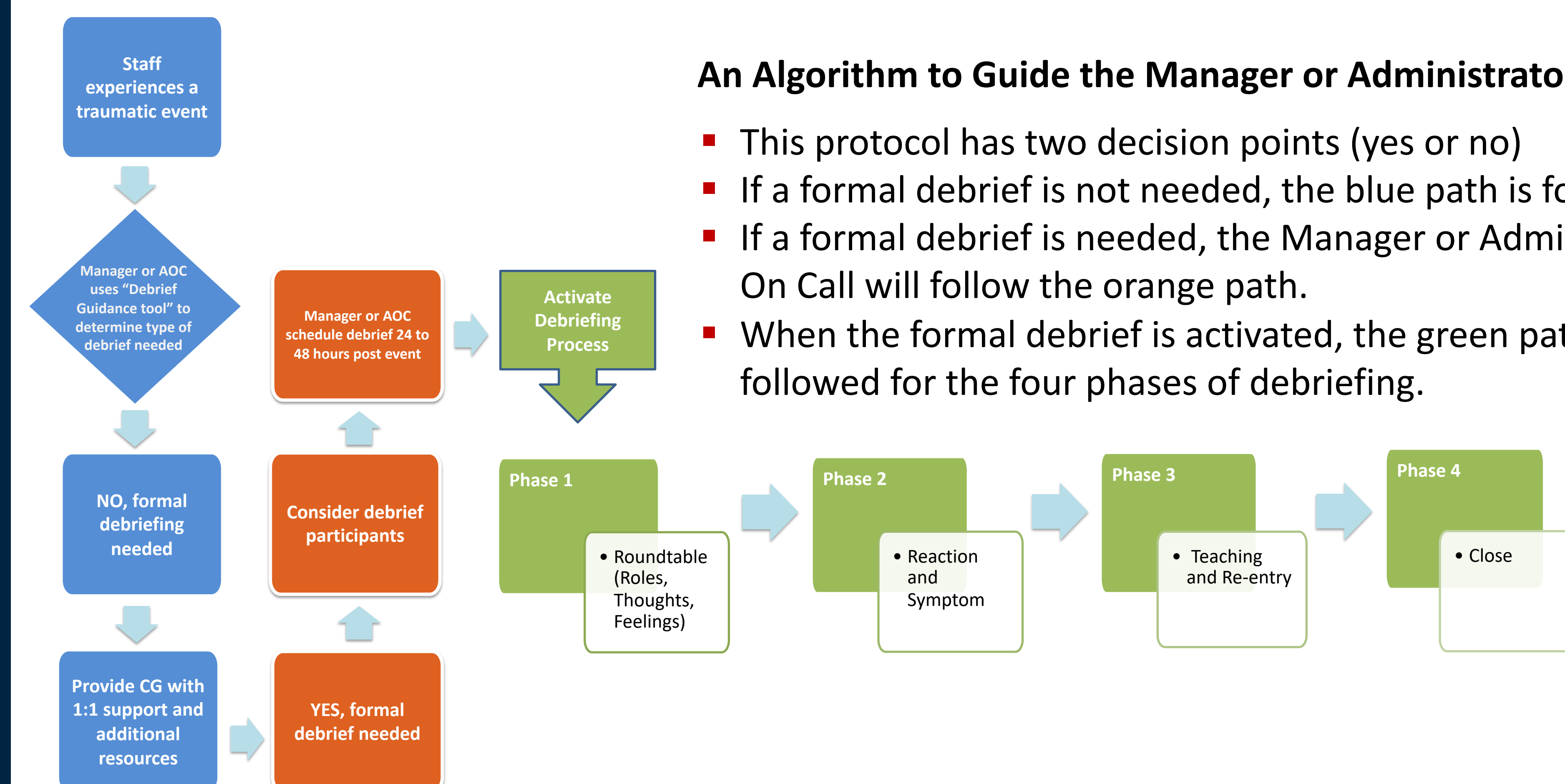


The Debrief Protocol



An Algorithm to Guide the Nurse

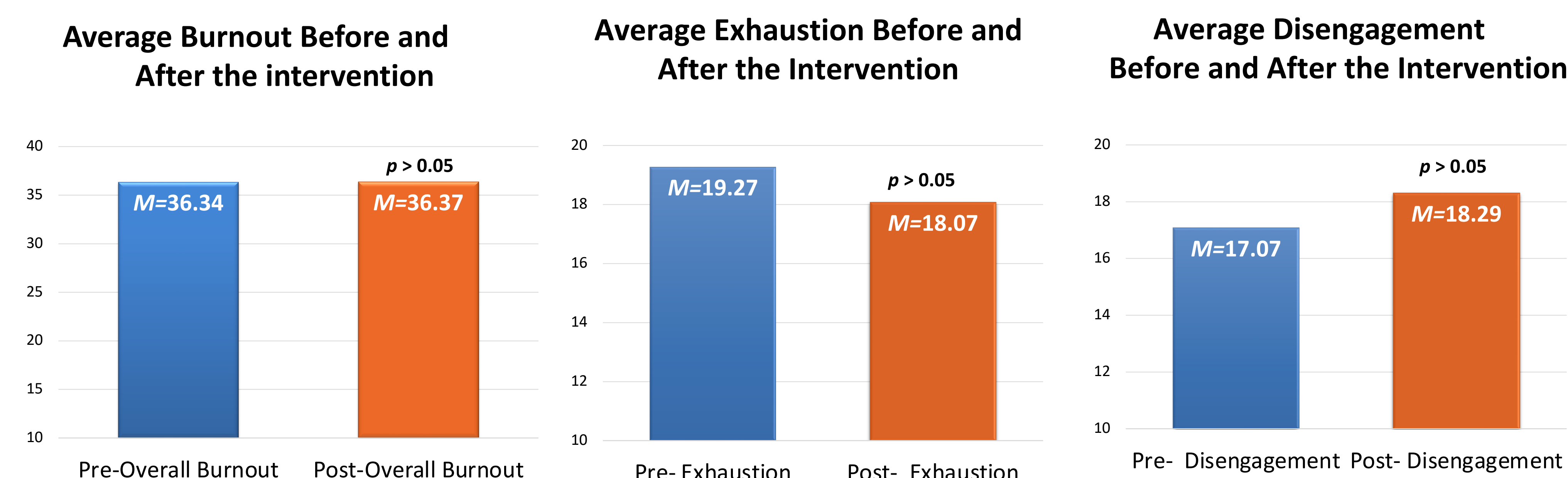
- This protocol guides nurses on how to activate support after a traumatic event.
- Explains an informal vs. formal debriefing.
- If a formal debriefing is needed, the four phases are explained to the nurse.
- A 30-60-90 post traumatic event occurs with manager.



An Algorithm to Guide the Manager or Administrator On Call

- This protocol has two decision points (yes or no)
- If a formal debrief is not needed, the blue path is followed.
- If a formal debrief is needed, the Manager or Administrator On Call will follow the orange path.
- When the formal debrief is activated, the green path will be followed for the four phases of debriefing.

Results



Methods

- Setting:** Hospice Agency in Southern California, with an average daily census of 290 patients.
- Design:** Pre-test Post-test design
- Sample:** 41 nurses including Registered Nurses and Licensed Vocational Nurses
- Measures:** Burnout, disengagement and exhaustion were measured using the Oldenburg Burnout Inventory (OLBI), Cronbach's alpha 0.81

Discussion/ Limitations

- Fidelity to Intervention Problem
 - Not enough time between education and training of intervention facilitators to the time of implementation, which might have affected how the intervention was delivered.
- General debriefing was occurring weekly for the pediatric hospice team at the same time; this might have created confusion among activating the debriefing protocol.
- Short Duration of the Intervention
 - These nurses have endured burnout for a long period of time. Their burnout is deep rooted, and this intervention may not have been adequate to attenuate it.

Conclusion

Interventions such as the debriefing protocol have been shown in literature to decrease burnout. However, the debriefing protocol used in this project did not attenuate burnout among hospice nurses in the duration the intervention was implemented.

Recommendation

- Retrain Managers, AOCs, and Nurses on the Debriefing Protocol.
- Retrain the Debriefing Protocol facilitators
- Observe and ensure that the intervention is delivered the way it is intended.
- Replicate study and use the OLBI for pre-test and post-test.
- Implement the intervention for a longer duration.

References

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